

PROPOSAL · PHASE ONE

SkinFit Wellness

Instant response, qualification, KAI brought into the WhatsApp journey, and a record of every conversation including calls.

01

What you told us

In our meeting Mr. Drish set out how he sees this, and what SkinFit actually needs at this stage. This proposal is scoped to that, not around it.

THE TWO PARTS

Marketing splits in two. Brand strategy is ours to own, and at this stage it is the priority. Optimisation and operations is where a partner is useful.

MR. DRISH ISRANI · OUR MEETING

WHAT IS NEEDED NOW

At today's volume the team has the bandwidth to handle operations. The optimisation layer starts adding real return closer to two thousand leads a month. What is needed now is speed to lead, qualification, and something that bridges brand strategy and the operational side, the calls and the texts.

MR. DRISH ISRANI · OUR MEETING

That is the specification, and this document answers it in that order. Speed to lead and qualification first. Then the bridge, which is the part nothing currently does: connecting what marketing spends to what actually gets said on the way to a booking.

Everything else stays out. Booking automation beyond a link, no-show recovery, quote follow-up and review generation are listed at the end as later decisions, to be made on evidence rather than assumption.

02

Your own numbers

You gave us these in the meeting. They answer the bandwidth question better than anything we could argue.

₹8,000 AD SPEND PER DAY	~60 LEADS PER DAY	~₹133 COST PER LEAD
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Across a month that is roughly **₹2.4 lakh into the top of the funnel** and **about 1,800 enquiries coming back out**. Taking the five percent you used in the meeting as an illustration, that is around three patients a day, and roughly ninety a month.

Which leaves about 1,700 people a month who message SkinFit and do not become patients. That is not a criticism of the team and not an argument that they lack bandwidth. Those conversations happen either way.

The question is what is left behind afterwards. Right now, nothing. Every objection raised, every reason someone gave for not booking, goes when the thread does. Three months of that is roughly five thousand conversations nobody can go back and read.

That matters here more than it would elsewhere, because of your own sequencing. Systems built later stand on the experience the team is building now, but only on the part of it that was actually recorded. Otherwise in three months there are the same anecdotes and still no data to build on.

Two things follow, and neither requires believing a projection.

- **One percentage point on 1,800 enquiries is about 18 more patients a month.** We are not promising a number. We are pointing out what a single point is worth at this volume.
- **Running this system costs less per month than four days of your current ad spend.** It sits on top of money you are already committed to spending.

And the volume is only going one way. You put the threshold at around two thousand leads a month, which at today's rate is close. **The layer that catches enquiries is cheaper to build before that point than after it**, because whatever the desk cannot catch at sixty a day it will not catch at a hundred.

03

What we would build

Three parts of one system, answering the three things you named.

01

ANSWERS: SPEED TO LEAD

Instant response and qualification

Every enquiry answered in seconds, on whichever channel it arrives on, at any hour including evenings and Sundays while the ads are still spending.

WHAT IT DOES

- Replies immediately with something useful, not a holding greeting
- Captures concern, treatment interest, area, duration, timeline and availability
- Answers general questions while the patient waits for a person
- Escalates anything clinical to a human. It never gives medical advice
- Hands the front desk a short summary, so nobody starts from “how can I help you”

02

ANSWERS: QUALIFICATION

KAI brought into the WhatsApp journey

Handoff mechanism to be confirmed with your technical team. See Section 10.

The free KAI assessment becomes the reason someone engages, instead of something they find on their own afterwards.

WHAT IT DOES

- Leads with the free assessment as the opening hook
- Frames the questions as helping KAI assess accurately, which raises completion
- Routes the patient into KAI with their context already captured
- Where KAI can return a result, brings it back into WhatsApp so follow-up is built on their own score

03

ANSWERS: THE BRIDGE

Conversation records

Every WhatsApp thread and every call tied to one patient record, searchable, with call transcripts.

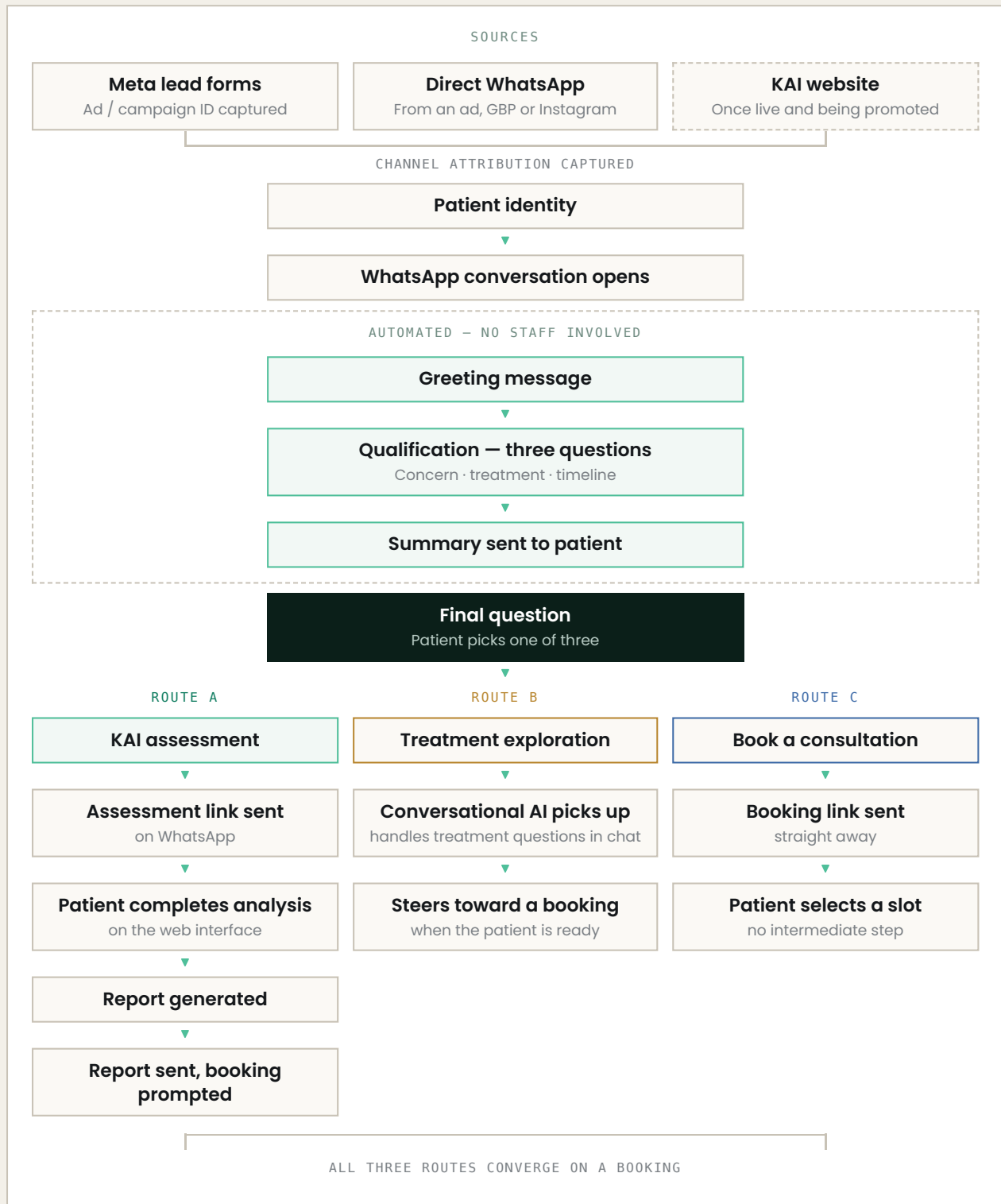
WHAT IT DOES

- Stores the full WhatsApp thread against the patient
- Records and transcribes calls between the clinic and the patient, with consent
- Attribution chain preserved end to end: source, KAI, conversation, booking
- Anyone can open a lead and read the whole history, across channels, in one view

04

One intake, three routes

Every lead lands in the same automated conversation, whatever the source. **Source is recorded for attribution, not for routing.**

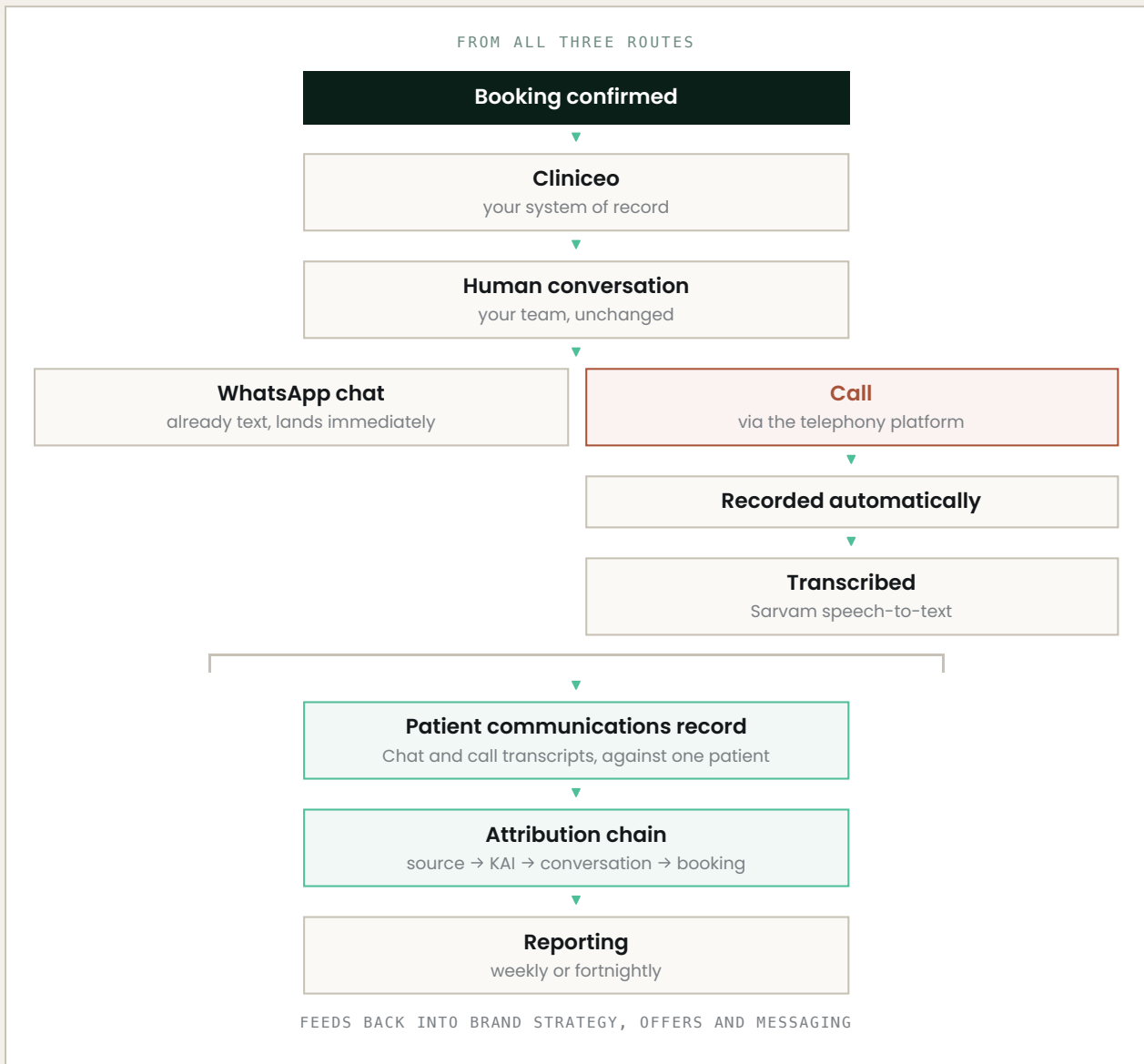


One intake, three exits. The greeting, the questions and the summary are identical for every lead. Only the final question branches, and every route still ends in the same place.

05

After the booking, and how the loop closes

All three routes converge on the same tail. This is where the record gets built, and where the feedback to marketing comes from.



This is the bridge. Brand strategy sits on one side, the calls and the texts sit on the other, and today nothing connects them. What gets said on the way to a booking never reaches the people deciding the offers.

We are not proposing to decide your offers or your positioning. That stays Part A and it stays yours. We build the measurement and feedback layer that gives you better information to make those calls with.

06

Why this is not a WhatsApp tool

You will have seen platforms that promise something similar. Three differences that matter.

INTEGRATION

Built around KAI, not around a template

A generic tool ships a template: greeting, menu, book. It has no concept of KAI, so the assessment becomes a link someone pastes into the chat. No context travels with the patient, no report comes back, and nothing ties the assessment to the booking that follows. **The one thing that makes the funnel yours ends up as an attachment.**

OPERATING

Built and run for you, not handed over

Grexa, Wati, AiSensy and Chatterbox all assume someone has time to build the flow and keep it current. At SkinFit that person is Mr. Gino, who is already running content and performance. So the flow gets built once, the offers move on, and it quietly stops matching what the clinic actually sells. **That is not a product defect. It is what happens when the tool is the deliverable.**

STRATEGIC

Operations that feed strategy

Ask any of them for last month's five most common objections, or which campaign produced a paying patient rather than a conversation. None can answer, and not because the data is missing. Reading the conversations was never the product. **They store. They do not read.**

07

What changes for your team, and what you will see

Very little changes, deliberately.

WHY THE DESK KEEPS THE CALLS

No-show recovery, the calls and the follow-ups stay with the front desk on purpose. The team needs ground-level experience of the operational work, speaking to patients and handling objections, because systems built later on top of that experience stand on a better foundation.

MR. DRISH ISRANI · OUR MEETING

We have built to that. The human sales conversation stays human.

- They stop typing the same first message sixty times a day
- They receive a short summary instead of a cold notification
- They work a ranked queue rather than a flat one
- They still make every call, handle every objection and close every consultation
- **No-show follow-ups, quote follow-ups, calls and review requests stay with them**, so the team keeps its on-ground experience with patients

Visible from month one, not in three: time to first response per channel and hour, qualification completion and where people drop off, how many complete a KAI assessment, the objections that recur most, and which campaigns produce enquiries that reach a booking.

Reporting stays deliberately descriptive at first. We show you what happened, not claim to explain why, until there is enough SkinFit data to say so honestly.

08

Why the call channel sits in a separate proposal

The other three systems need nothing from the clinic beyond access. **This one asks the team to change how it makes phone calls**, every call, every day. That is an operational commitment rather than a technical one, which is why it is scoped, priced and decided on its own.

THE CONSTRAINT EVERYTHING FOLLOWS FROM

Voice only works if calls run through the telephony platform, which means reps dial from a desktop softphone rather than a handset. A call made from a personal phone bypasses the platform entirely and produces no recording, no transcript, and no record that it happened.

WHAT SKINFIT WOULD BE COMMITTING TO

- **Desk-bound calling.** Reps dial from a computer, not their phones, for every patient call
- **A telephony account in the clinic's name**, with numbers provisioned through it
- **A virtual number**, which changes the number patients see and call back on
- **Recording disclosure to patients** before a recording starts, under the DPDP Act
- **Someone owning that account** and its usage billing
- **Accepting the gap:** any call placed off-platform leaves no trace in the record

WHAT HAS TO BE BUILT AND WIRED TOGETHER

- Telephony integration and number provisioning
- Softphone set up on each rep's machine, and the calling habit that goes with it
- **Recording retrieval** that pulls each file before its link expires
- **Speech-to-text** through Sarvam, then speaker separation so rep and patient are distinguishable
- Attaching each transcript to the correct patient record
- Premium-tier API access for programmatic retrieval

The risk worth naming. If desk-bound calling turns out to be unrealistic on a clinic floor, the voice branch quietly stops capturing most calls, and the record looks complete while being partial. That is worse than not having it.

Nothing in the other three systems depends on this. **It can be added at any point** once the base is running and the team has decided the calling change is workable.

11

Investment

Build and setup

₹50,000

One time, for everything in this proposal

Monthly operation and optimisation

₹27,000

Begins once the first system is live

EXCLUSIVE OF GST. USAGE BILLED SEPARATELY AT COST, AS SET OUT BELOW.

The monthly fee covers keeping the system running and correct: platform and template management as offers change, refinement of the qualification and KAI sequences against real drop-off, handling the edge cases that surface once patients are actually using it, and the reporting described earlier.

These are firm figures, not estimates. The one thing that could move them is if the KAI handoff turns out to work differently from what your team expects. If that happens we would agree any change with you before it affects anything above.

What the system runs on, and who pays for what

Set out in full so nothing arrives later as a surprise. **Usage is billed at cost. We do not mark it up.**

COMPONENT	ROLE	HOW IT IS CHARGED
Meta / WhatsApp	Message delivery and channel intake	Variable, billed at cost
Sarvam	Language, qualification and voice transcription	Variable, billed at cost
Exotel	Inbound and outbound call handling	Variable, billed at cost

The application layer, workflow state, logs and reporting are included in the monthly fee. Cliniceo stays on your existing subscription.

MESSAGE USAGE, FOR PLANNING

INTERACTIONS A MONTH	META	SARVAM	COMBINED
1,000	₹115	₹66	₹181
10,000	₹1,150	₹659	₹1,809

VOICE USAGE, IF THE CALL CHANNEL IS BUILT

INBOUND VOICE	SARVAM VOICE	EXOTEL	COMBINED
Per minute	₹3.50	₹0.30 to ₹0.50	₹3.80 to ₹4.00
500 minutes a month	₹1,750	₹150 to ₹250	₹1,900 to ₹2,000

09

Data and consent

- 01 **Patient records stay where they are.** We work on incoming messages, not medical records.
- 02 **Nothing automated ever gives medical advice.** Clinical questions go to your team, always.
- 03 **Call recording requires consent.** A short disclosure before recording begins, and patients can decline.
- 04 **Only people who contacted you first are messaged,** within what WhatsApp and Meta permit. No purchased lists.
- 05 **Access and data stay yours.** Numbers and accounts in the clinic's name, access withdrawable at any time, no patient data taken elsewhere.

10

What we would align on with your team

Your technical team is already building the KAI website, Cliniceo can be integrated against, and Cliniceo support handles onboarding. So most of this is coordination rather than open questions.

- **The KAI handoff.** How a patient passes into the assessment with their context attached, and how the report comes back into the thread. Your team owns the website side, we own the WhatsApp side
- **Cliniceo.** Confirming what the integration allows on bookings, with their onboarding support
- **Two design choices.** Where treatment exploration hands off, and whether the KAI report goes out as one message or two
- **The 60 to 65 figure.** Whether that counts every raw enquiry or only qualified ones

Booking automation beyond the link, no-show recovery, quote follow-up and review generation stay out of scope, matching your view that those are worth building once there is data behind them.

What happens next. Put us in touch with whoever is building the KAI website. We will work the handoff out with them directly and come back with the two design choices settled. No meeting needed at any point.

The timing does matter slightly. The handoff is far easier to design into the website while it is being built than to retrofit once it is live, so the useful window is while your team still has it open.

Commercials follow once that shape is settled, so what you are quoted matches what is actually being built.